

SKIN & LASER

Laser Resurfacing Protocol

Ablative and non-ablative fractional resurfacing: selection, prophylaxis, anesthesia and wound care

DOCUMENT SKN-005	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Authorized laser operators and supervising clinicians	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Resurfacing is the highest-risk elective service in a skin practice. It creates a controlled wound over a large surface area, with real infection, scarring and pigmentary risk, and a recovery the patient must be prepared for before they consent. This protocol covers modality selection, prophylaxis, anesthesia planning, and day-by-day wound care.

AUTHORIZATION REQUIRED

Ablative resurfacing is performed only by operators specifically authorized for it on the competency matrix, with the supervising clinician available as defined by the medical director and your state's rules. Being signed off for laser hair removal or non-ablative treatment does not authorize ablative resurfacing.

2 Modality Selection

FRAMEWORK — CONFIRM AGAINST YOUR DEVICE IFU

Modality	Typical wavelength	Suited to	Considerations
Non-ablative fractional	1540 / 1550 nm	Texture, mild scarring, fine lines, patients who cannot take downtime.	Several sessions required. Lower risk. Safer across a wider range of skin types.
Non-ablative, superficial targeting	1927 nm	Pigmentation, actinic damage, tone.	Minimal dermal effect. Good pigment tool with modest downtime.
Ablative fractional	2940 nm (Er:YAG) / 10,600 nm (CO2)	Deeper wrinkling, acne scarring, significant photodamage.	Substantial downtime. Higher infection, PIH and scarring risk. Er:YAG is less thermally aggressive than CO2.
Fully ablative	CO2 or Er:YAG, non-fractional	Severe photodamage and scarring.	Longest recovery, highest risk. Only where explicitly authorized; many practices refer these.

SKIN TYPE AND DENSITY

Post-inflammatory hyperpigmentation risk rises sharply with Fitzpatrick type and with treatment density. In Fitzpatrick III and above, reduce density before reducing depth, extend intervals, prime the skin, and plan more sessions. Fitzpatrick V–VI ablative resurfacing requires explicit medical director authorization case by case.

3 Screening, Priming & Prophylaxis

- ☐ Fitzpatrick assessment, photography and test area per SKN-007.
- ☐ Isotretinoin within the interval set by the medical director — the most commonly cited resurfacing contraindication.
- ☐ HSV history anywhere on the face — prophylaxis required regardless of treatment zone.
- ☐ Keloid or hypertrophic scarring history.
- ☐ Active infection, acne, or inflammatory dermatosis in the field.
- ☐ Immunosuppression, poorly controlled diabetes, smoking, or impaired healing.
- ☐ Prior radiation to the area, or prior surgical undermining such as a facelift within a defined interval.
- ☐ Connective tissue or photosensitive disease.
- ☐ Recent sun exposure or tanning.
- ☐ Realistic downtime discussion — with dates. Patients must not book resurfacing shortly before an event.
- ☐ Priming regimen completed and compliance confirmed.
- ☐ Post-procedure support confirmed: someone to drive them, and to help at home if needed.

Prophylaxis	Standard
Antiviral	Required for all facial resurfacing where HSV history exists, and commonly for all patients regardless of history for ablative treatment. Started before the procedure and continued through re-epithelialization. Agent, dose and duration set by the medical director.
Antibacterial	Not routine. Considered case by case for extensive ablative treatment or where risk factors exist. Prescriber decision, documented with reasoning.
Antifungal	Considered for extensive ablative treatment or where candidal infection risk is raised. Prescriber decision.

HERPETIC DISSEMINATION

HSV reactivation across freshly resurfaced skin can be severe, scarring and disseminated. Prophylaxis is started before treatment, not at the first sign of a lesion. If the patient did not start it, reschedule.

4 Anesthesia Planning

CROSS-REFERENCE LAST

Resurfacing anesthesia frequently combines a compounded topical over a large surface area with injected local — the exact stacking that causes local anesthetic systemic toxicity. Calculate and record the total dose in milligrams against the patient's weight before you begin, and follow EMG-007 in the Emergency Protocols kit. Lipid emulsion availability is a medical director decision that must be made before this service is offered.

- Record the patient's weight and calculate the maximum permitted local anesthetic dose before any anesthetic is applied.
- Log the running total, including anesthetic contained within any other product used.
- Topical anesthetic applied to the smallest necessary area, for a timed interval, with occlusion only where explicitly authorized in writing.
- Set a timer. Remove on schedule. Never leave a patient unattended with a large-area occluded topical.

- Remove all topical anesthetic completely before treatment begins.
- Nerve blocks and injected local administered only by authorized personnel.
- Monitor for early toxicity throughout — metallic taste, tinnitus, perioral numbness, agitation — and stop immediately if any appear.

5 Treatment & Immediate Post-Procedure

- 1 Full laser safety setup** per SKN-008: eyewear, controlled area, signage, plume evacuation, fire precautions. Ablative resurfacing carries genuine ignition risk — no alcohol-based preps, no supplemental oxygen, wet drapes.
- 2 Confirm authorization, consent, prophylaxis compliance and anesthesia totals.**
- 3 Photograph** from standardized angles before treatment.
- 4 Map the treatment zones** and record the parameters planned for each. Feathering at borders prevents demarcation lines.
- 5 Confirm density, depth and passes** against the IFU. Density drives downtime and PIH risk more than depth does.
- 6 Treat zone by zone**, watching tissue response continuously. Wipe debris per IFU between passes where directed.
- 7 Stop immediately** on greying, charring, pinpoint bleeding beyond the expected endpoint, or loss of the expected tissue response.
- 8 Cool the skin** and apply the post-procedure occlusive specified.
- 9 Observe before discharge** and confirm the patient has their aftercare, medications, contact numbers and a driver.
- 10 Record** zones, parameters, density, passes, endpoints and total anesthetic dose.

6 Wound Care & Follow-Up

Phase	Expected	Care
First 24–48 hours	Marked erythema, oedema, oozing, heat and discomfort.	Frequent gentle cleansing with the specified solution. Occlusive emollient kept in place continuously. Cool compresses. Head elevated when sleeping.
Days 2–5	Peak swelling, then crusting and sloughing.	Continue cleansing and occlusion. No picking. Antiviral continued. Daily contact from the practice.
Re-epithelialization	New pink epithelium as crusting separates.	Transition to bland emollient. Introduce sun protection as soon as tolerated. Still no actives.
Weeks 2–8	Prolonged erythema, gradually fading. Peak PIH risk window.	Strict sun avoidance. Consider pigment-directed therapy per the medical director. Reintroduce actives only when authorized.

ESCALATE THE SAME DAY

Pain that increases rather than settles. Spreading erythema beyond the treated field, purulence, fever or malaise. Grouped vesicles or punched-out erosions suggesting HSV. Any area failing to re-epithelialize on schedule, or appearing grey, dusky or ulcerated. Delayed re-epithelialization is the earliest warning sign of scarring.

- ☐ Daily contact for the first several days after ablative treatment.
- ☐ In-person review at re-epithelialization and again at the interval set by the medical director.
- ☐ Standardized photography at every review.
- ☐ Sun protection and pigment management reinforced at every contact.
- ☐ No further resurfacing until fully healed and reviewed.
- ☐ Any complication photographed, escalated and reviewed by the medical director.

7 Quick Reference

QUICK REFERENCE CARD

Resurfacing essentials

1. **Authorized operator only.** Check the competency matrix.

2. **Antiviral started before treatment.** Not started → reschedule.

3. **Calculate anesthetic in mg against weight.** Follow EMG-007.

4. **Fire risk:** no alcohol prep, no supplemental oxygen, wet drapes.

5. **Density drives PIH and downtime** more than depth. Reduce it first.

6. **Feather the borders.** Demarcation lines are permanent-looking.

7. **Daily contact for the first several days.**

8. **Delayed re-epithelialization = early scarring.** Escalate same day.

Print, laminate and post at the point of care

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8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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